

JOHNS HOPKINS NEUROLOGY REFERRAL COVER SHEET

Fax received: 03/11/2026 14:22 EST (quality: fair, some cutoff text on right margin)

Referring Clinic: Green Valley Internal Medicine

Referring Provider: Michael T. Renshaw, MD

Phone: (703) 555-1842

Fax: (703) 555-1903

Patient Name: Harold J. Whitcomb

DOB: 02/17/1948

MRN (outside): GV-883920

Reason for Referral:

“Gait instability, frequent falls, r/o neuropathy vs central cause vs deconditioning”

Requested Service:

General Neurology (check)

Movement Disorders (handwritten circle, unclear if intentional)

Urgency: Routine (but see note: “fall risk worsening?” written in margin)

Insurance: Medicare + supplemental (see attached)

Additional Comments:

“Progressive difficulty walking past year. PT not helping. Occasional tremor? Family concerned re cognition vs balance.”

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PATIENT DEMOGRAPHICS (exported from EHR, print view)

Name: Harold James Whitcomb

Sex: M

DOB: 02/17/1948 (Age 78)

Address: 14219 Cedar Brook Ln, Centreville, VA 20120

Phone: (571) 555-2289

Emergency Contact: Linda Whitcomb (spouse) (571) 555-2210

Primary Language: English

Marital Status: Married

Occupation: Retired accountant

Insurance:

Medicare Part B

Aetna Supplement

Height: 5'9"
Weight: 174 lb (last recorded 01/2026)
BMI: 25.7

Allergies:
NKDA (one note says "PCN rash as child?" unclear)

Pharmacy: CVS Centreville Rd

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REFERRING PROVIDER NOTE

Date: 02/28/2026

Provider: Michael T. Renshaw, MD

Chief Complaint: Falls

HPI:

78-year-old male with PMH HTN, HLD, T2DM, BPH, GERD, chronic low back pain presents with progressive gait instability over ~12–18 months. Patient reports "legs not doing what I want," especially when turning. Wife notes shuffling and occasional "freezing," though patient denies this specifically. Has had 3–4 falls in past 6 months, one resulting in minor wrist sprain (see ED note attached).

Denies vertigo. Reports mild dizziness when standing quickly. No syncope.
Denies focal weakness but says "slower overall."

There is mention of intermittent tremor of right hand at rest per spouse, not consistently observed in clinic.

Cognitive concerns: wife notes "forgetfulness," but MMSE in office 27/30 (missed recall items).

Urinary urgency present x several years (BPH vs other?).

ROS otherwise negative except fatigue and constipation.

Exam (limited):

BP 132/78 sitting

Gait: slow, slightly wide-based (?uncertain), decreased arm swing noted but patient holding jacket

Romberg: negative

Strength 5/5 throughout

Reflexes 2+ symmetric

No clear tremor seen

Tone not formally assessed

Assessment:

Progressive gait instability with falls. Differential includes peripheral neuropathy (diabetic), lumbar spinal stenosis, normal pressure hydrocephalus (though no clear triad), early parkinsonism, or general deconditioning.

Plan:

Refer to neurology
MRI brain (ordered, see report)
Labs pending
Continue PT

Signed electronically

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PRIOR NOTE #1 (PRIMARY CARE FOLLOW-UP)

Date: 10/14/2025

Provider: M. Renshaw, MD

Chief Complaint: "Balance off"

HPI (copy-forwarded sections present):

Patient reports feeling "off balance" for several months. No falls at that time. Notes difficulty initiating walking occasionally but attributes to back stiffness.

Denies tremor. (NOTE: conflicting with later note)

Reports numbness in feet "for years," consistent with diabetic neuropathy.

Exam:

Gait: mildly cautious
Monofilament decreased in bilateral feet
No tremor
Normal strength

Assessment:

Likely multifactorial gait disturbance (neuropathy + age-related changes).

Plan:

PT referral
Check B12
Optimize diabetes control

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PRIOR NOTE #2 (URGENT CARE VISIT)

Date: 12/22/2025

Facility: Fairfax Urgent Care

Chief Complaint: Fall

HPI:

Patient fell while turning in kitchen, states “feet got stuck.” Denies LOC. No head strike.

Wife reports he has been “slowing down” and sometimes has a slight shaking of hands when sitting.

Exam:

Gait observed as shuffling with reduced step height

Mild resting tremor R hand observed intermittently

No focal deficits

Assessment:

Mechanical fall vs neurologic gait disorder. Consider Parkinsonian features vs medication effect.

Plan:

Recommend neurology follow-up

No imaging performed

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PRIOR NOTE #3 (ORTHOPEDICS SPINE CLINIC)

Date: 01/19/2026

Chief Complaint: Low back pain, gait issues

HPI:

Patient referred for evaluation of possible lumbar stenosis contributing to gait difficulty. Reports leg fatigue when walking distances. Denies radicular pain.

States gait “gets stuck sometimes,” especially in tight spaces (NOTE: wording unclear if patient volunteered vs prompted).

Exam:

Lumbar ROM limited

Straight leg raise negative

Gait: slow, stooped posture noted (unclear if pain-related)

Imaging reviewed: lumbar MRI (see below) shows moderate L4-L5 stenosis

Assessment:

Lumbar stenosis may contribute but does not fully explain symptoms.

Plan:

Conservative management

Consider neurology

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LAB RESULTS (mixed panel)

Collected: 02/25/2026

CBC:

WBC 6.2

Hgb 13.1

Hct 39.2

Platelets 210

CMP:

Na 139

K 4.2

Cr 1.0

Glucose 142 (fasting unclear)

HbA1c: 7.4%

TSH: 2.1

Vitamin B12: 412

Folate: normal

ESR: mildly elevated 28

UA: negative

Lipid panel attached (irrelevant)

Total cholesterol 168

LDL 92

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IMAGING REPORTS

MRI BRAIN WITHOUT CONTRAST

Date: 02/27/2026

Findings:

Mild generalized cerebral volume loss

Periventricular white matter changes consistent with chronic microvascular ischemia

Ventricles mildly prominent but proportionate to atrophy (radiologist notes "NPH less likely but clinical correlation recommended")

No acute infarct

Impression:

Chronic small vessel ischemic changes

No acute pathology

MRI LUMBAR SPINE

Date: 01/10/2026

Findings:

Moderate central canal stenosis at L4-L5

Mild degenerative changes elsewhere

Impression:

Degenerative lumbar stenosis

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MEDICATION LIST (reconciled 02/2026, may be incomplete)

Metformin 1000 mg BID

Lisinopril 20 mg daily

Atorvastatin 40 mg nightly

Tamsulosin 0.4 mg daily

Omeprazole 20 mg daily

Aspirin 81 mg daily

Gabapentin 300 mg TID (for neuropathy)

Cyclobenzaprine PRN (rare use)

Vitamin D 1000 IU daily

Multivitamin

Melatonin PRN sleep

Note: One prior list included "propranolol?" but patient denies current use

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EMERGENCY DEPARTMENT DISCHARGE SUMMARY

Date: 01/03/2026

Facility: Inova Fair Oaks Hospital

Reason for Visit: Fall with wrist pain

Summary:

Patient presented after fall at home. States he “lost balance while pivoting.” No LOC.

Exam:

Vitals stable

Neurologic exam nonfocal

Gait not formally assessed

X-ray wrist: no fracture

Disposition:

Discharged home

Advised follow-up with PCP

Additional note:

“Patient appears slow in movements but oriented x3.”

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PHYSICAL THERAPY INITIAL EVALUATION

Date: 11/02/2025

Subjective:

Patient reports imbalance and difficulty walking quickly. Denies tremor.

Objective:

Gait speed reduced

Short step length

Difficulty with turns (takes multiple small steps)

Assessment:

Balance impairment likely multifactorial

Plan:

Gait training, strengthening

Progress note (12/2025 addendum):

Limited improvement. Therapist notes “possible neurologic component?”

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